

Medicare Benefits Schedule - Item 2715

Search Results for Item 2715

[View Associated Notes](#)

Category 1 - PROFESSIONAL ATTENDANC

2715 ⓘ (.popoverContent)

Group

A20 - GP Mental Health Treatment

Subgroup

1 - GP Mental Health Treatment Plans

Professional attendance by a general practitioner (including a general practitioner who has undertaken mental health skills training of at least 20 minutes but less than 40 minutes in duration for the preparation of a GP mental health treatment plan for a patient)

Fee: \$108.95 **Benefit:** 75% = \$81.75 100% = \$108.95

(See para [AN.0.56](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.0.56)(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.0.56>), [AN.0.78](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.0.78)

(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.0.78>), [AN.36.2](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.36.2)(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&qt=NoteID&q=AN.36.2>) of explanatory notes to this Category)

Extended Medicare Safety Net Cap: ⓘ \$326.85

[← Previous - Item 2701\(fullDisplay.cfm?type=item&q=2701&qt=ItemID\)](#)

[Next - Item 2717 → \(fullDisplay.cfm?type=item&q=2717&qt=ItemID\)](#)

Associated Notes

Category 1 - PROFESSIONAL ATTENDANC

GP Mental Health Treatment Plans and Consultation

Mental Health Treatment Plans and Consultation Items

Associated items: 272, 276, 281, 282, 2700, 2701, 2715, 2717, 92112, 92113, 92114, 92117, 92118, 92119, 92122 and 92123.

This note provides information on the preparation and review of Mental Health Treatment Plans under the Better Access initiative by a general practitioner (GP) or prescribed medical practitioner (PMP) at the patient's MyMedicare registered practice or by their usual medical practitioner (refer to explanatory note [AN.0.78 – Better Access Initiative](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.78&qt=noteID&criteria=AN%2E0%2E78) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.78&qt=noteID&criteria=AN%2E0%2E78>)).

Who can provide

The Mental Health Treatment Plan Medicare Benefits Schedule (MBS) items are available for use by a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner

Under MyMedicare and usual medical practitioner requirements, GPs and PMPs will be able to access time-tiered professional (general) attendance items to review a Mental Health Treatment Plan and refer and/or provide ongoing management of a patient's mental health condition, recognising the time spent with patients to provide quality care. There will be no limitation on the number of health-related issues that can be addressed as part of a general attendance consultation, if the requirements of the service are met, and adequate and contemporaneous records are maintained. Key principles for using time-tiered professional (general) attendance items and any specific requirements is available at [AN.0.9 | Using time-tiered professional \(general\) attendance items](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9>).

Patient eligibility

These services are available to patients who meet the relevant eligibility requirements (refer to explanatory note [AN.0.78 – Better Access Initiative](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.78&qt=noteID&criteria=AN%2E0%2E78) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.78&qt=noteID&criteria=AN%2E0%2E78>)) undertaken by either a GP or PMP at the general practice in which the patient is enrolled in MyMedicare, or regardless of whether the patient is enrolled in MyMedicare, by the patient's usual medical practitioner. This also includes a GP or PMP who is located at a medical practice that has provided the majority of their care over the previous 12 months or will be providing the majority of their care over the next 12 months. These services can also be provided to private inpatients (including private inpatients who are residents of aged care facilities) being discharged from hospital. Where the service is provided as part of an episode of hospital treatment, the service must be claimed at the 75% MBS benefit.

Services attracting Medicare benefits

The Mental Health Treatment Plan items define services for which Medicare benefits are payable where a patient's GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner to undertake early intervention, assessment, and management of patients with diagnoseable mental disorders. For information on the service requirements of these items, please see the relevant headings below.

There are 16 MBS items for the preparation of a Mental Health Treatment Plan. These are:

For GPs providing Mental Health Treatment Plans who have undertaken mental health skills training recognised through the General Practice Mental Health Standards Collaboration:

- Face-to-face items: 2715 and 2717
- Video items 92116 and 92117

For GPs who have not undertaken mental health skills training:

- Face-to-face items 2700 and 2701
- Video items 92112 and 92113

For PMPs providing Mental Health Treatment Plans who have undertaken mental health skills training recognised through the General Practice Mental Health Standards Collaboration:

- Face-to-face items 281 and 282

- Video items 92122 and 92123

For PMPs who have not undertaken mental health skills training:

- Face-to-face items 272 and 276
- Video items 92118 and 92119

It is strongly recommended that GPs and PMPs providing mental health treatment have appropriate mental health training. GP organisations support the value of appropriate mental health training for practitioners using these items.

There are items that exist for the review of Psychiatrist Assessment and Management Plans. For further information, refer to explanatory note [AN.0.30 – Consultant Psychiatrist – Referred Patient Assessment and Management Plan](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30)(<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30>).

Preparation of a Mental Health Treatment Plan

A Mental Health Treatment Plan is intended to identify and document the care needs of patients with a clinically diagnosed mental disorder to allow for a structured approach to the management of their treatment. For the purposes of Better Access services, a mental disorder is defined as a significant impairment of an individual's cognitive, affective and/or relational abilities which may require intervention and may be a recognised, medically diagnosable illness or disorder. This definition is informed by the World Health Organisation, 1996, Diagnostic and Management Guidelines for Mental Disorders in Primary Care: ICD-10 Chapter V Primary Care Version. For the purposes of Better Access services, dementia, delirium, tobacco use disorder and mental retardation are not regarded as mental disorders.

Before proceeding with a Mental Health Treatment Plan service, the GP or PMP at the patient's MyMedicare registered practice or their usual medical practitioner must ensure that:

- the Mental Health Treatment Plan preparation service and the steps involved in preparing a Mental Health Treatment Plan are explained to the patient and (if appropriate and with the patient's permission) to the patient's carer; and
- the patient's agreement to proceed is recorded.

The preparation of a Mental Health Treatment Plan involves the assessment of a patient's mental disorder and preparation of a plan to assist the patient in managing their mental disorder.

The preparation of a Mental Health Treatment Plan must be documented in writing, and include an assessment process consisting of:

- the administration of an outcome measurement tool (except if considered clinically inappropriate); and
- the formulation of the mental disorder, including a provisional or formal diagnosis.

An outcome measurement tool refers to a tool used to monitor changes in a patient's health that occur in response to treatment received by the patient.

The choice of the evidence-based outcome measurement tool/s to be used is at the clinical discretion of the practitioner, however examples of appropriate outcome tools include:

- Kessler Psychological Distress Scale (K10)
- DASS 21 (Depression, Anxiety and Stress)

GPs using such tools should be familiar with their appropriate clinical use, and if not, should seek appropriate education and training.

Once a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner has completed an assessment of a patient's mental disorder, the following must be undertaken:

- identify and discuss referral and treatment options with the patient, including appropriate support services;
- agree upon treatment goals with the patient and any actions the patient will take;
- provide relevant and suitable education about the patient's mental disorder;
- create a plan for crisis intervention and/or for relapse prevention; and
- make arrangements for required referrals, treatment, appropriate support services, review and follow-up.

This information must be recorded in writing in the patient's Mental Health Treatment Plan. The Initial Assessment and Referral Decision Support Tool (IAR-DST) can be used by the GP or PMP at the patient's MyMedicare registered practice or their usual medical practitioner. It helps to determine the most appropriate level of mental health care needed and informs a suitable referral decision. The IAR-DST aligns with the assessment components required in a patient's Mental Health Treatment Plan and can be incorporated during its preparation. This enables streamlined referrals to mental health services, many of which require a completed IAR at the point of entry. Referrals for treatment services under the Better Access initiative should be utilised for patients who require at least a moderate level of support.

Upon completion of the preparation of a Mental Health Treatment Plan, a copy must be offered to the patient, and where relevant and if the practitioner considers it appropriate, the patient's carer, before a Mental Health Treatment Plan item may be claimed. A copy of the Mental Health Treatment Plan must be added to the patient's medical records.

A patient's Mental Health Treatment Plan should be treated as a living document for updating as required. In particular, the plan can be updated at any time to incorporate relevant information, such as feedback or advice from other health professionals on the diagnosis or treatment of the patient. A new plan should not be prepared unless clinically required, and generally not within 12 months of a previous plan, unless exceptional circumstances exist, e.g. if the patient has had a significant change to their mental health. Ongoing mental health consultations and reviews of progress can be provided by the GP or PMP through the time-tiered professional (general) attendance MBS items, as required.

MBS Telehealth (video) services for Mental Health Treatment Plans

MyMedicare and usual medical practitioner requirements apply to GP and PMP telehealth (video) items for Mental Health Treatment Plans, with these services not exempt from the MBS telehealth eligibility criteria. For further information on the MBS telehealth eligibility criteria, refer to explanatory note [AN.1.1](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.1.1&qt=noteID&criteria=established%20clinical%20relationship%20>).

Referral and treatment options

Eligible patients must be referred to eligible treatment services by either a GP or PMP at the general practice in which the patient is enrolled in MyMedicare, or regardless of whether the patient is enrolled in MyMedicare, by the patient's usual medical practitioner who is managing the patient under a Mental Health Treatment Plan. This also includes a GP or PMP who is located at a medical practice that has provided the majority of their care over the previous 12 months or will be providing the majority of their care over the next 12 months. Eligible patients may also be referred for Better Access treatment services under a Psychiatrist Assessment and Management Plan, or on direct referral from a psychiatrist or paediatrician.

Once a Mental Health Treatment Plan has been completed and claimed on an appropriate Medicare service, a patient is eligible to access relevant treatment and referral options. Medicare benefits are available for up to 10 individual and 10 group therapy mental health treatment services in a calendar year. The services may consist of:

- psychological therapy services delivered by eligible clinical psychologists (refer explanatory note [MN.6.2 - Provision of Psychological Therapy](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.2&qt=noteID&criteria=mn%2E6%2E2>)); and/or
- focussed psychological strategies services delivered by eligible GPs, eligible PMPs, eligible psychologists (registered), eligible social workers and eligible occupational therapists (refer to explanatory note [MN.7.4 – Provision of Focussed Psychological Strategies](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.4&qt=noteID&criteria=MN%2E7%2E4>)).

The referring practitioner can decide how many individual treatment services the patient will receive in a course of treatment, within the maximum service limit for the course of treatment. The maximum service limit for each course of treatment is set out below:

- Initial course of treatment - a maximum of 6 individual services.
- Subsequent course of treatment - remaining individual services up to the patient's cap of 10 services per calendar year (for example, if the patient received 6 services in their initial course of treatment, they could only receive 4 services in a subsequent course of treatment provided within the same calendar year).
- Up to a maximum of 10 group therapy mental health treatment services per calendar year

The referring practitioner must specify on the referral the number of services to be provided as part of the course of treatment. Once a course of treatment is undertaken, a review of the patient's Mental Health Treatment Plan is required for the patient to access an additional course of treatment.

For additional information on Better Access referral requirements, please see explanatory note [MN.6.3 - Referral requirements for Better Access Treatment Services](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.3&qt=noteID&criteria=MN%2E6%2E3%20>).

Reviewing a Mental Health Treatment Plan

A review of a patient's Mental Health Treatment Plan, including referral of treatment services and ongoing management of a patient's mental healthcare, is a key component for assessing and managing the patient's progress. A Mental Health Treatment Plan should be reviewed at least once a year by a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner using time-tiered professional (general) attendance MBS items. Key principles for using time-tiered professional (general) attendance items and any specific requirements is available at explanatory Note [AN.0.9 | Using time-tiered professional \(general\) attendance items](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9) (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9).

Once a systematic review of the patient's progress against the Mental Health Treatment Plan review requirements has been completed, a Medicare benefit can be claimed using the time-tiered professional (general) attendance items. Where a psychiatrist has prepared a referred Psychiatrist Assessment and Management Plan (item 291 or 92435), as if that patient had a Mental Health Treatment Plan, the dedicated MBS items for a review of a Psychiatrist Assessment and Management Plan (items 293, 296, 297, 299 or 92436 or 92437) can be used.

Before proceeding with any Mental Health Treatment review service, the GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner must ensure:

- the Mental Health Treatment Plan review service and the steps involved are explained to the patient and (if appropriate and with the patient's permission) to the patient's carer; and
- the patient's agreement to proceed is recorded.

The review, undertaken using time-tiered professional (general) attendance MBS items, must include:

- a review of the patient's progress against the goals outlined in the Mental Health Treatment Plan;
- checking, reinforcing and expanding education;
- a plan for crisis intervention and/or for relapse prevention, if appropriate and if not previously provided;
- re-administration of the outcome measurement tool used in the assessment stage, except where considered clinically inappropriate; and
- modification of the documented Mental Health Treatment Plan if required.

A medical practitioner may administer an appropriate triage tool, such as the IAR-DST, to determine the most appropriate level of mental health care required. This tool can also support and inform a practitioner's referral decision.

If amendments are made to the patient's Mental Health Treatment Plan, a copy of the amended plan must be offered to the patient, and where relevant and appropriate, the patient's carer, before a Mental Health Treatment Plan review service (undertaken using time-tiered professional [general] attendance MBS items), may be claimed. A copy of the amended plan must also be added to the patient's medical records. GPs and PMPs are able to address a number of health-related issues, in addition to reviewing a Mental Health Treatment Plan, as part of using time-tiered professional (general) attendance MBS items, as long as the requirements of the service are met, and adequate and contemporaneous records are maintained.

In general, most patients should not require more than 2 time-tiered professional (general) attendance items, for the specific purpose of reviewing a Mental Health Treatment Plan in a calendar year. A review of a Mental Health Treatment Plan should not be undertaken more than once in a 3-month period, or within 4 weeks following a claim for a Mental Health Treatment Plan item. When reviewing a Mental Health Treatment Plan, any referrals for additional treatment services under the Better Access initiative should be utilised for patients who require at least a moderate level of support. Consideration should be given to other treatment interventions and pathways (e.g. digital mental health services, Medicare Mental Health Centres, etc) should the patient's level of support no longer require psychological intervention under the Better Access initiative.

Patients will be required to obtain a new referral from their referring practitioner after they have received the number of services stated on the referral, or the maximum number of services (1 group therapy mental health treatment services) allowed in a calendar year.

Mental health treatment consultation

For an extended consultation with a patient where the primary treating problem is related to a mental disorder, including for a patient being managed under a Mental Health Treatment Plan, time-tiered professional (general) attendance items can be used. These items should not be used for the initial development of a Mental Health Treatment Plan. Time-tiered professional (general) attendance items should only be used for Mental Health Treatment Plan review services and to deliver mental health care and support to patients.

A Mental Health Treatment Consultation must include:

- taking relevant history and identifying the patient's presenting problem(s) (if not previously documented);
- providing treatment, advice and/or referral for other services or treatment;
- documenting the outcomes of the consultation in the patient's medical records and other relevant mental health plan (where applicable); and

- any other requirement set out in the item descriptor for the time-tiered professional (general attendance) items.

A patient may be referred from this consultation for other treatment and services. This does not include referral for Better Access services, unless the patient is being managed by a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner under a Mental Health Treatment Plan or under a Psychiatrist Assessment and Management Plan (item 291 or 92435). A direct referral may also be made by a psychiatrist or paediatrician.

Additional claiming information

The time-tiered professional (general) attendance items should be used to review a Mental Health Treatment Plan and deliver mental health care and support to patients. There will be no limitation on the number of health-related issues that can be addressed in the time-tiered professional (general) attendance items, as long as the requirements of the service are met (as part of a review of a Mental Health Treatment Plan or a Mental Health Treatment Consultation) and adequate and contemporaneous records are maintained. For information on what constitutes adequate and contemporaneous records, refer to explanatory note [GN.15.39 - Practitioners should maintain adequate and contemporaneous records](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=GN.15.39&qt=noteID&criteria=GN%2E15%2E39>). To co-claim a Mental Health Treatment Plan MBS item and another attendance item, both services must be clinically relevant and distinct services.

Where an additional consultation is undertaken, both services must be clinically relevant, and all item requirements must be met which are set out in the item descriptor for the time-tiered professional (general attendance) items. For example, for item 2700, the duration of the service must have been at least 20 minutes. The time of the preceding consultation must not be counted towards the time of the mental health service.

All consultations conducted as part of the Mental Health Treatment Plan must be rendered by a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner. A specialist mental health nurse, other allied health professionals, Aboriginal and Torres Strait Islander health practitioner or Aboriginal and Torres Strait Islander health worker with appropriate mental health qualifications and training may provide assistance to GPs in provision of mental health care, however, for a Medicare benefit to be payable all requirements of the item must be met.

Further information

For further information on the Better Access Initiative, refer to explanatory note [AN.0.78 – Better Access Initiative](#) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.78&qt=noteID&criteria=AN%2E0%2E78>).

For further information about Medicare Benefits Schedule items, please visit the MBS online website at www.health.gov.au/mbsonline .

Further information is available for providers from the Services Australia provider enquiry line on 132 150.

If you are a patient seeking advice about Medicare services, patient benefits, or your Medicare claims, please contact Services Australia on the Medicare General enquiry line on 132 011.

Related Items: [272](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=272>) [276](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=276>) [281](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=281>) [282](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=282>) [2700](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=2700>) [2701](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=2701>) [2715](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=2715>) [2717](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=2717>) [92112](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92112>) [92113](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92113>) [92116](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92116>) [92117](#) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92117>)

Category 1 - PROFESSIONAL ATTENDANCE

AN.0.78

Better Access Initiative

Summary

This explanatory note provides an overview of The Better Access to Psychiatrists, Psychologists and General Practitioners through the Medicare Benefits Schedule initiative (Better Access), which commenced on 1 November 2006. Under this initiative, Medicare benefits are available to patients for selected mental health treatment services provided by eligible general practitioners (GPs), eligible prescribed medical practitioners (PMPs), psychologists (eligible clinical and eligible registered), eligible social workers and eligible occupational therapists.

Further information on the eligibility requirements of a GP can be found at 2.20.7 (Restrictions on items in Subgroup 2 of Group A20 [focussed psychological strategies]) and a PMP can be found at 2.20.7A (Restrictions on items in Subgroup 9 of Group A7 [focussed psychological strategies]). The [Health Insurance \(General Medical Services Table\) Regulations 2021](#) [↗] provides further information on the legislative requirements for eligible GPs and PMPs. For further information on the qualification requirements for Allied Health Professionals who are able to provide mental health services under Medicare, refer to Schedule 1 (Qualification requirements for allied health professionals) in the [Health Insurance \(Section 3C General Medical Services – Allied Health and other Primary Health Care Services\) Determination 2024](#) [↗].

All Medicare Benefits Schedule (MBS) explanatory notes that are associated with the Better Access initiative are listed below:

Explanatory Note	Relevant item numbers
AN.0.56 - Mental Health Treatment Plans and Consultation (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.56&qt=noteID&criteria=AN%2E0%2E56)	272, 276, 277, 282, 2700, 2701, 2715, 2717, 92112, 92113, 92116, 92117, 92118, 92119, 92122, and 92123.
MN.6.2 - Provision of Psychological Therapy (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.2&qt=noteID&criteria=mn%2E6%2E2)	80000, 80005, 80010, 80015, 80020, 80021, 80022, 80023, 80024, 80025, 91166, 91167, 91181, and 91182.
MN.7.4 - Provision of Focussed Psychological Strategies (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.4&qt=noteID&criteria=MN%2E7%2E4)	283, 285, 286, 287, 2721, 2723, 2725, 2727, 80100, 80105, 80110, 80115, 80120, 80121, 80122, 80123, 80125, 80127, 80128, 80130, 80135, 80140, 80145, 80146, 80147, 80148, 80150, 80152, 80153, 80155, 80160, 80165, 80170, 80171, 80172, 80173, 80174, 80175, 91818, 91819, 91842, 91843, 91820, 91821, 91844, 91845, 91169, 91170, 91183, 91184, 91172, 91173, 91185, 91186, 91175, 91176, 91187 and 91188.
MN.6.3 - Referral requirements for Better Access Treatment Services (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.3&qt=noteID&criteria=MN%2E6%2E3%20)	272, 276, 281, 282, 283, 285, 286, 287, 2700, 2701, 2715, 2717, 2721, 2723, 2725, 2727, 80000, 80005, 80010, 80015, 80020, 80021, 80022, 80023, 80024, 80025, 80100, 80105, 80110, 80115, 80120, 80121, 80122, 80123, 80125, 80127, 80128, 80130, 80135, 80140, 80145, 80146, 80147, 80148, 80150, 80152, 80153, 80155, 80160, 80165, 80170, 80171, 80172, 80173, 80174, 80175, 91167, 91169, 91170, 91172, 91173, 91175, 91176, 91181, 91182, 91183, 91184, 91185, 91186, 91187, 91188, 91818, 91819, 91820, 91821, 91842, 91843, , 91844, 91845, 92112, 92113, 92116, 92117, 92118 and 92119. <i>Note: for specialist psychiatrists and paediatricians these services include any of the specialist attendance items 104 through 109 and items 91822, 91823 and 91833. For consultant physician psychiatrists the relevant eligible Medicare services cover any of the consultant psychiatrist items 293 to 308, 310, 312, 314, 316, 318 or 319 through 349; while for consultant physician paediatricians the eligible services are consultant physician attendance items 110 through 133.</i>
AN.15.1 - Mental Health Case Conferences (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.15.1&qt=noteID&criteria=AN%2E15%2E1)	930, 933, 935, 937, 943, 945, 946, 948, 959, 961, 962, 964, 969, 971, 972, 973, 975, 986, 80176, 80177 and 80178.
MN.7.5 - Family and Carer Participation (https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.5&qt=noteID&criteria=MN%2E7%2E5)	309, 311, 313, 315, 2739, 2741, 2743, 2745, 80002, 80006, 80012, 80016, 80102, 80106, 80112, 80116, 80129, 80131, 80137, 80141, 80154, 80156, 80162, 80166, 91859, 91168, 91171, 91174, 91177, 91194, 91195, 91196, 91197, 91198, 91199, 91200, 91201, 91202, 91203, 91204, 91205, 91861, 91862, 91863, 91864, 91865, 91866, and 91867.

Services provided under Better Access

Through Better Access, eligible patients can claim a Medicare benefit for up to 10 individual and 10 group therapy mental health treatment services per calendar year. These services consist of:

- psychological therapy provided by eligible clinical psychologists (refer to explanatory note [MN.6.2 - Provision of Psychological Therapy](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.2&qt=noteID&criteria=MN%2E6%2E2) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.2&qt=noteID&criteria=MN%2E6%2E2>)); and
- focussed psychological strategies provided by eligible GPs, eligible PMPs, eligible psychologists, eligible occupational therapists, and eligible social workers (refer to explanatory note [MN.7.4 – Provision of Focussed Psychological Strategies](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.4&qt=noteID&criteria=MN%2E7%2E4) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.4&qt=noteID&criteria=MN%2E7%2E4>)).

A Medicare benefit is also available for a patient to involve another person, such as a family member or carer, in their treatment. Medicare benefits are available to a patient for up to 2 services provided to another person per calendar year. Any services delivered using these MBS items count towards the patient's course of treatment and calendar year allocations under Better Access. For further information on involving another person in a patient's treatment, refer to explanatory note [MN.7.5 – Family and Carer Participation](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.5&qt=noteID&criteria=MN%2E7%2E5) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.7.5&qt=noteID&criteria=MN%2E7%2E5>).

There are also MBS items for the provision of mental health case conferencing services to establish and coordinate the management of the care needs of a patient. For further information refer to explanatory note [AN.15.1– Mental Health Case Conferences](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.15.1&qt=noteID&criteria=AN%2E15%2E1) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.15.1&qt=noteID&criteria=AN%2E15%2E1>).

Patient eligibility for Better Access services

Better Access services apply to people with a clinically diagnosed mental disorder. For the purposes of Better Access services, a mental disorder is defined as a significant impairment of an individual's cognitive, affective and/or relational abilities which may require intervention and may be a recognised, medically diagnosable illness or disorder. This definition is informed by the World Health Organisation, 1996, Diagnostic and Management Guidelines for Mental Disorders in Primary Care: ICD-10 Chapter V Primary Care Version. For the purposes of Better Access services, dementia, delirium, tobacco use disorder and mental retardation are not regarded as mental disorders.

When referring for treatment services under the Better Access initiative, services should be utilised for patients who require at least a moderate level of mental health support. Consideration should be given to other treatment interventions and pathways (e.g. digital mental health services, Medicare Mental Health Centres, etc) should the patient's level of support not require psychological intervention under the Better Access initiative.

Referral requirements

Additionally, Better Access MBS treatment items will only attract a Medicare benefit where a patient has been referred for Better Access services by a GP or PMP at the patient's MyMedicare registered practice or by their usual medical practitioner, or by a specialist or consultant physician specialising in the practice of their field of psychiatry or paediatrics. For the purposes of these services, referring practitioner means:

- *usual medical practitioner*, in relation to a patient, means a general practitioner or prescribed medical practitioner:
 - (a) who has provided the majority of services to the person in the past 12 months; or
 - (b) who is likely to provide the majority of services to the person in the following 12 months; or
 - (c) located at a medical practice that:
 - (i) has provided the majority of services to the person in the past 12 months; or
 - (ii) is likely to provide the majority of services to the person in the next 12 months.
- A GP or PMP at a MyMedicare registered practice means the registration program by that name administered by the department. A MyMedicare service means a service to which an item is provided:
 - (a) to a person enrolled in MyMedicare; and
 - (b) at the general practice at which the person is enrolled.
- a medical practitioner who has referred the patient as part of a GP Mental Health Treatment Plan (refer to explanatory note [AN.0.56 – GP Mental Health Treatment Plans and Consultation](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.56&qt=noteID&criteria=AN%2E0%2E56) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.56&qt=noteID&criteria=AN%2E0%2E56>)) or a Psychiatrist Assessment and Management

Plan (refer to explanatory note [AN.0.30 - Consultant Psychiatrist – Referred Patient Assessment and Management Plan](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30)

(<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30>)); or

- a specialist or consultant physician specialising in the practice of their field of psychiatry directly referring for Better Access services; or
- a specialist or consultant physician specialising in the practice of their field of paediatrics directly referring for Better Access services.
- the patient referral requirements, as part of a Mental Health Treatment Plan (refer to explanatory note [AN.0.56 – Mental Health Treatment Plans and Consultation Items](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.56&qt=noteID&criteria=AN%2E0%2E56) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.56&qt=noteID&criteria=AN%2E0%2E56>)) or a Psychiatrist Assessment and Management Plan (refer to explanatory note [AN.0.30 - Consultant Psychiatrist – Referred Patient Assessment and Management Plan](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.30&qt=noteID&criteria=AN%2E0%2E30>)), have been met.

Focussed psychological strategies are available to any patient from any GP or PMP who has the appropriate training recognised by the General Practice Mental Health Standards Collaboration. GPs and PMPs who provide focussed psychological strategies do so as part of an arrangement for the treatment of an assessed mental disorder under a Mental Health Treatment Plan.

For more information on referral requirements for Better Access services, refer to explanatory note [MN.6.3 - Referral requirements for Better Access Treatment Services](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.3&qt=noteID&criteria=MN%2E6%2E3%20)

(<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=MN.6.3&qt=noteID&criteria=MN%2E6%2E3%20>).

If there is any doubt about a patient's eligibility, Services Australia will be able to confirm whether a Mental Health Treatment Plan and/or a Psychiatrist Assessment and Management Plan is in place and claimed; or an eligible psychiatric or paediatric service has been claimed, as well as the number of Better Access services already claimed by the patient during the calendar year. Additionally, providers can also access the Health Professional Online Services (HPOS) to view a patient's Mental Health Treatment Plan history.

Allied health professionals can call Services Australia on 132 150 to check this information, while unsure patients can seek clarification by calling 132 011 or view their care plan history in their Medicare online account through myGov to help track services that have been claimed.

A patient will not be eligible to receive a Medicare benefit if they have not been appropriately referred or if a relevant Medicare service has not been provided to them. If the referring service has not yet been claimed, Services Australia will not be aware of the patient's eligibility. In this case the treating practitioner or allied health professional should, with the patient's permission, contact the referring practitioner to ensure a relevant referring service has been provided to the patient.

Mental Health Treatment Plan review requirements

After a patient has used the allocated number of services in the initial individual course of treatment (a maximum of 6 services on the initial referral), a review of the patient's Mental Health Treatment Plan and a new referral must be obtained from the referring practitioner for further treatment. The review will assess the patient's progress and consider further treatment options. A review is not to be undertaken more than once in a 3 month period or within 4 weeks following the preparation of a Mental Health Treatment Plan unless exceptional circumstances exist. It is recommended a Mental Health Treatment Plan be reviewed at least once during a course of treatment, with most patients generally not requiring more than 2 reviews in a calendar year.

GPs and PMPs are to use time tiered professional (general attendance) items for the specific purpose of reviewing a Mental Health Treatment Plan. A Mental Health Treatment Plan does not expire. A new Mental Health Treatment Plan should not be created unless exceptional circumstances exist. An exceptional circumstance may be where the patient has had a significant change to their mental health or the treating practitioner or treating allied health professional is unable to obtain a copy of their Mental Health Treatment Plan.

In addition to the 10 individual services, a patient can be referred up to a maximum of 10 group therapy mental health treatment services each calendar year on any one referral. Patients will be required to have a review of their Mental Health Treatment Plan and obtain a new referral from their referring practitioner after they received the number of group therapy mental health treatment services stated on the referral (if less than 10).

The referring practitioner will use their clinical discretion when undertaking a review to determine the number of allied mental health services their patient will be referred for and if the patient requires additional MBS mental health services. Medicare benefits are only available for a maximum of 10 individual and 10 group therapy mental health treatment services each calendar year. Patients can privately fund or use other funding sources they are eligible for to access further services.

Under MyMedicare and usual medical practitioner requirements, GPs and PMPs will be able to access time-tiered professional (general) attendance items to review and provide mental health care to patients recognising the time spent with patients to provide quality care. There will be no limitation on the number of health-related issues that can be addressed as part of a general attendance consultation, as long as the requirements of the service are met, and adequate and contemporaneous records are maintained. Key principles for using time-tiered professional (general) attendance items and any specific requirements is available at [AN.0.9 | Using time-tiered professional \(general\) attendance items](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=AN.0.9&qt=noteID&criteria=AN%2E0%2E9>).

Out-of-pocket expenses and Medicare safety net

Charges in excess of the Medicare benefit for Better Access MBS items are the responsibility of the patient. However, any out-of-pocket costs will count towards the Medicare safety net for that patient. The out-of-pocket costs for mental health services which are not Medicare eligible do not count towards the Medicare safety net (refer to explanatory note [GN.10.27 | Medicare Safety Nets](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=GN.10.27&qt=noteID&criteria=GN%2E10%2E27) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=GN.10.27&qt=noteID&criteria=GN%2E10%2E27>)).

Publicly funded services

Better Access MBS items do not apply for services that are provided by any other Commonwealth or state funded services or provided to an admitted patient of a hospital. Unless an exemption under subsection 19(2) of the Health Insurance Act 1973 applies for a Medicare benefit to be payable, all requirements of the service must be met, including registration with Services Australia.

Private health insurance

Patients need to decide if they will use Medicare or their private health insurance ancillary cover to pay for these services. Patients cannot use their private health insurance ancillary cover to 'top up' the Medicare benefit paid for the services. If a patient has exhausted their services for which a Medicare benefit is payable for the calendar year, and if they have appropriate private health insurance which covers psychological services, they may claim from their private health fund. The benefit payable to members of private health funds will vary based on the level of their ancillary cover.

Record keeping and reporting requirements

The Department of Health, Disability and Ageing undertakes regular post payment auditing to ensure MBS items are claimed appropriately. All practitioners providing Better Access services should ensure they keep adequate and contemporaneous records, including documenting the date, time and people who attended. Only clinical details recorded at the time of attendance count towards the time of the consultation. Other notes or reports added at a later time are not included. For information on what constitutes adequate and contemporaneous records, refer to explanatory note [GN.15.39 - Practitioners should maintain adequate and contemporaneous records](https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=GN.15.39&qt=noteID&criteria=GN%2E15%2E39) (<https://www9.health.gov.au/mbs/fullDisplay.cfm?type=note&q=GN.15.39&qt=noteID&criteria=GN%2E15%2E39>).

Relevant Legislation

Details about the legislative requirements of the Better Access initiative MBS items can be found on the Federal Register of Legislation at www.legislation.gov.au [↗](#).

Attendance items are set out in 3 regulatory instruments:

- [*Health Insurance \(Section 3C General Medical Services – Allied Health and other Primary Health Care Services\) Determination 2024*](#) [↗](#) - Eligible psychologist (clinical and registered), eligible social worker, and eligible occupational therapist items.
- [*Health Insurance \(General Medical Services Table\) Regulations 2021*](#) [↗](#) - General practitioner, prescribed medical practitioner and psychiatrist items, including items related to Focussed Psychological Strategies for eligible general practitioners and eligible prescribed medical practitioners.
- [*Health Insurance \(Section 3C General Medical Services – Telehealth Attendances\) Determination 2021 - telehealth \(video and phone\) items*](#) [↗](#) - General practitioner, prescribed medical practitioner, eligible psychologists (clinical and registered), eligible social worker, and eligible occupational therapist items. In addition, items related to focussed psychological strategies for eligible general practitioners and eligible prescribed medical practitioners are included in this instrument.

Further information

For further information about Medicare Benefits Schedule items, please visit the MBS Online website at www.health.gov.au/mbsonline [↗](#).

For providers, further information is also available from the Services Australia provider enquiry line on 132 150.

If you are a patient seeking advice about Medicare services, patient benefits, or your Medicare claims, please contact Services Australia on the Medicare General enquiry line on 132 011.

Related Items: [272](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=272) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=272>) [276](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=276) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=276>) [281](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=281) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=281>) [282](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=282) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=282>) [283](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=283) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=283>) [285](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=285) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=285>) [286](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=286) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=286>) [287](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=287) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=287>) [309](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=309) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=309>) [311](http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=311) (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=311>) [313](#)

[illegible]

(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91187>) **91188** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91188>) **91198**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91198>) **91199** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91199>) **91200**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91200>) **91201** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91201>) **91202**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91202>) **91203** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91203>) **91204**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91204>) **91205** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91205>) **91842**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91842>) **91843** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91843>) **91844**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91844>) **91845** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91845>) **91864**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91864>) **91865** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91865>) **91866**
(<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91866>) **91867** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=91867>)

Category 1 - PROFESSIONAL ATTENDANC

AN.36.2

Eating Disorders Treatment and Management Plans Explanatory Notes

Eating Disorders Treatment and Management Plans Explanatory Notes (items 90250-90257, 92146-92153, 90260-90261, and 92162-92163)

This note provides information on Eating Disorders Treatment and Management Plan (EDTMP) items and should be read in conjunction with the Eating Disorders General Explanatory Notes:

Eating Disorder Treatment and Management Plan (EDTMP) items overview

The EDTMP items define services for which Medicare benefits are payable where practitioners undertake the development of a treatment and management plan for patients with a diagnosis of anorexia nervosa, bulimia nervosa, binge-eating disorder and other specified feeding or eating disorder diagnoses who meet the eligibility criteria.

The EDTMP items trigger eligibility for items which provide delivery of eating disorders psychological treatment services (up to a total of 40 psychological services in a 12-month period) and dietetic services (up to a total of 20 in a 12-month period).

For any particular patient, an eating disorder treatment and management plan expires at the end of a 12-month period following provision of that service. Eating Disorders treatment services are not available to the patient if the EDTMP has expired.

Preparation of the EDTMP must include:

- discussing the patient's medical and psychological health status with the patient and if appropriate their family/carer;
- identifying and discussing referral and treatment options with the patient and their family/carer where appropriate, including identification of appropriate support services;
- agreeing goals with the patient and their family/carer where appropriate - what should be achieved by the treatment - and any actions the patient will take;
- planning for the provision of appropriate patient and family/carer education;
- a plan for crisis intervention and/or for relapse prevention, if appropriate at this stage;
- making arrangements for required referrals, treatment, appropriate support services, review and follow-up;
- documenting the results of assessment, patient needs, goals and actions, referrals and required treatment/services, and review date in the patient's plan;
- discussing and organising the appropriate reviews throughout the patient's treatment; and
- discussing the need for the patient to be reviewed to access a higher intensity of eating disorder psychological treatment services in a 12-month period.

Preparing a Medical practitioner in general practice Eating Disorder Treatment & Management Plan (items 90250-90257 and 92146-92153)

Who can provide the service

Items in subgroup 1 of Group A36 can be rendered by a medical practitioner in general practice. This includes:

- Medical practitioners who can render a general practitioner service in Group A1 of the MBS (see note AN.0.9 for the types of medical practitioners). These medical practitioners can render a 'general practitioner' service for items in subgroup 1 of Group A36.
- Medical practitioners who are not general practitioners, specialists or consultant physicians. These medical practitioners can render a 'medical practitioner' service for items in subgroup 1 of Group A36.

What is Involved - Assess and Plan

It is expected that the practitioner developing the EDTMP has either performed or reviewed the assessments and examinations required to make a judgement that the patient meets the eligibility criteria for accessing these items.

Items 90250-90257 and their equivalent telehealth items (92146-92153) provide services for development of the eating disorder treatment and management plan. Where a comprehensive physical examination is performed, either on the same occasion or different occasion, the appropriate item could be claimed provided the time taken performing the assessment is not included in the time for producing the plan, or time producing the EDTMP is not included in the time for assessment.

It is emphasised that it is best practice for the practitioner to perform a comprehensive physical assessment to facilitate ongoing patient management and monitoring of medical and nutritional status.

Patient Assessment

An assessment of a patient with an eating disorders includes:

- taking relevant history (biological, psychological, social, including family/carer support);
- eating disorder diagnostic assessment;
- medical review including physical examination and relevant tests;
- conducting an assessment of mental state, including identification of comorbid psychiatric conditions;
- an assessment of eating disorder behaviours;
- an assessment of associated risk and any medical co-morbidity, including as assessment on how this impacts on the patients functioning and activities of daily living;
- an assessment of family and/or carer support; and
- administering an outcome measurement tool, except where it is considered clinically inappropriate.

Risk assessment for a patient with an eating disorder should include identification of:

- medical instability and risk of hospitalisation;
- level of psychological distress and suicide risk;
- level of malnourishment;
- identification of psychiatric comorbidity;
- level of disability;
- duration of illness;
- response to earlier evidence-based eating disorders treatment;
- level of family/carer support.

It should be noted that the patient's EDTMP should be treated as a living document for updating as required. In particular, the plan can be updated at any time to incorporate relevant information, such as feedback or advice from other health professionals on the diagnosis or treatment of the patient.

Preparing a Consultant Psychiatrist Eating Disorder Treatment & Management Plan (90260-90261 and 92162-92163)

Who can provide the service

Items in subgroup 2 of Group 36 can be rendered by consultant psychiatrists (items 90260 and 90261, and their respective telehealth items 92162 and 92163).

What is Involved – Assess and Plan

Items 90260-90261 and their equivalent telehealth items (92162 and 92163) provide access to specialist assessment and treatment planning. It is expected that items will be a single attendance. However, there may be particular circumstances where a patient has been referred by a GP for an assessment and management plan, but it is not possible for the consultant to determine in the initial consultation whether the patient is suitable for management under such a plan. In these cases, where clinically appropriate, other appropriate consultation items may be used. In those circumstances where the consultant undertakes a consultation (in accordance with the item requirements) prior to the consultation for providing the referring practitioner with an assessment and management plan. It is expected that such occurrences would be unusual for the purpose of diagnosis under item 90260.

Patient Assessment

In order to facilitate ongoing patient focussed management, an assessment of the patient must include:

- administering an outcome measurement tool during the assessment and review stages of treatment, where clinically appropriate. The choice of outcome tool to be used is at the clinical discretion of the practitioner;
- conducting a mental state examination;
- taking relevant history (biological, psychological, behavioural, nutritional, social);
- assessing associated risk and any co-morbidity; and
- making a psychiatric diagnosis for conditions meeting the eligibility criteria.

Risk assessment for a patient with an eating disorder should include identification of:

- medical instability and risk of hospitalisation;
- level of psychological distress and suicide risk;
- level of malnourishment;
- identification of psychiatric comorbidity;
- level of disability;
- duration of illness;
- response to earlier evidence-based eating disorders treatment;
- level of family/carer support.

Where a consultant psychiatrist provides an EDTMP service, the service must also include:

- administering an outcome measurement tool, where clinically appropriate. The choice of outcome tool to be used is at the clinical discretion of the practitioner. Practitioners using such tools should be familiar with their appropriate clinical use, and if not, should seek appropriate education and training; and
- conducting a mental state examination.

Consultation with the patient's managing practitioner

A written copy of the EDTMP should be provided to the patient's managing practitioner, within a maximum of two weeks of the assessment. It should be noted that two weeks is the outer limit and in more serious cases more prompt provision of the plan and verbal communication with the managing practitioner may be appropriate.

Additional Claiming Information (general conditions and limitations)

Patients seeking benefits for items 90250-90257 and 90260-90261 will not be eligible if the patient has had a claim within the last 12-months.

Items 90250-90257 cannot be claimed with Items 2713, 279, 735, 758, 235 and 244. Items 90261 cannot be claimed with Items 110, 116, 119, 132, 133.

Consultant psychiatrist and paediatrician EDTMP items 90260-90261 do not apply if the patient does not have a referral within the period of validity.

Before proceeding with the EDTMP the medical practitioner must ensure that:

- (a) the steps involved in providing the service are explained to the patient and (if appropriate and with the patient's permission) to the patient's carer; and
- (b) the patient's agreement to proceed is recorded.

The medical practitioner must offer the patient a copy of the EDTMP and add the document to the patient's records. This should include, subject to the patient's agreement, offering a copy to their carer, where appropriate. The medical practitioner may, with the permission of the patient, provide a copy of the EDTMP, or relevant parts of the plan, to other providers involved in the patient's treatment.

The medical practitioner EDTMP cover the service of developing an EDTMP. A separate consultation item can be performed with the EDTMP if the patient is treated for an unrelated condition to their eating disorder. Where a separate consultation is performed, it should be annotated separately on the patient's account that a separate consultation was clinically required/indicated.

All consultations conducted as part of the EDTMP must be rendered by the medical practitioner and include a personal attendance with the patient. A specialist mental health nurse, other allied health practitioner, Aboriginal and Torres Strait Islander health practitioner or Aboriginal and Torres Strait Islander health worker with appropriate mental health qualifications and training may provide general assistance to the medical practitioner in provision of this care.

Additional Claiming Information (interaction with Chronic Condition Management and Better Access)

It is preferable that wherever possible patients have only one plan for primary care management of their disorder. As a general principle the creation of multiple plans should be avoided, unless the patient clearly requires an additional plan for the management of a separate medical condition.

The Chronic Condition Management (CCM) items (items 231, 232, 392, 393, 729, 731, 965, 967, 92026, 92027, 92029, 92030, 92057, 92058, 92060 and 92061) continue to be available for patients with chronic medical conditions, including patients with complex needs.

Where a patient has a separate chronic medical condition, it may be appropriate to manage the patient's medical condition through a CCM Plan, and to manage their eating disorder through an EDTMP. In this case, both items can be used. Where the patient receives dietetic services under the CCM arrangements (item 10954), these services will count towards the patients maximum of 20 dietetic services in a 12-month period.

Where a patient has other psychiatric comorbidities, these conditions should be managed under the EDTMP. Once a patient has a claim for an EDTMP, the patient should not be able to have a claim for the development or review of a Mental Health Treatment plan by a GP (items 2700, 2701, 2715 and 2717) or medical practitioner in general practice (items 272, 276, 281 and 282) within 12-months of their EDTMP unless there are exceptional circumstances.

For the purpose of the 40 eating disorder psychological treatment count; eating disorder psychological treatment service includes a service provided under the following items: 90271-90278, 92182, 92184, 92186, 92188, 92194, 92196, 92198, 92200, 2721, 2723, 2725, 2727, 283, 285, 286, 287 and items in Groups M6, M7 and M16 (excluding item 82350).

Related Items: **110** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=110>) **116** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=116>) **119** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=119>) **132** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=132>) **133** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=133>) **231** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=231>) **232** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=232>) **235** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=235>) **244** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=244>) **272** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=272>) **276** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=276>) **281** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=281>) **282** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=282>) **283** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=283>) **285** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=285>) **286** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=286>) **287** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=287>) **392** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=392>) **393** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=393>) **729** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=729>) **731** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=731>) **735** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=735>) **758** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=758>) **965** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=965>) **967** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=967>) **2700** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2700>) **2701** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2701>) **2715** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2715>) **2717** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2717>) **2721** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2721>) **2723** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2723>) **2725** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2725>) **2727** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=2727>) **82350** (<http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&q=ItemID&q=82350>) **90250**

(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90250)	<u>90251</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90251)	<u>90252</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90252)	<u>90253</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90253)	<u>90254</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90254)	<u>90255</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90255)	<u>90256</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90256)	<u>90257</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90257)	<u>90260</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90260)	<u>90261</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90261)	<u>90271</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90271)	<u>90272</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90272)	<u>90273</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90273)	<u>90274</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90274)	<u>90275</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90275)	<u>90276</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90276)	<u>90277</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90277)	<u>90278</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90278)	<u>90266</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90266)	<u>90207</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90207)	<u>90209</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90209)	<u>90230</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=90230)	<u>90257</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=902057)	<u>902058</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=902058)	<u>90260</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=902060)	<u>902061</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=902061)	<u>92146</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92146)	<u>92147</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92147)	<u>92148</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92148)	<u>92149</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92149)	<u>92150</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92150)	<u>92151</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92151)	<u>92152</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92152)	<u>92153</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92153)	<u>92162</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92162)	<u>92163</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92163)	<u>92182</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92182)	<u>92184</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92184)	<u>92186</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92186)	<u>92188</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92188)	<u>92194</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92194)	<u>92196</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92196)	<u>92198</u>
(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92198)	<u>92200</u>	(http://www9.health.gov.au/mbs/fullDisplay.cfm?type=item&qt=ItemID&q=92200)	

Legend

Assist - Addition/Deletion of (Assist.)

Amend - Amended Description

Anaes - Anaesthetic Values Amended

Emsn - EMSN Change

Fee - Fee Amended

Renum - Item Number Change (renumbered)

New - New Item

NewMin - New Item (previous Ministerial Determination)

Qfe - QFE Change

 [Subscribe](#)

HOME

[Accessibility](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/Accessibility)(http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/Accessibility)

[Privacy](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/privacy_security)(http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/privacy_security)

[Disclaimer](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/disclaimer)(http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/disclaimer)

[Copyright](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/copyright)(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/copyright>)

THE MBS

[Search](#)([search.cfm](#))

[Create publication](#) ([createPublication.cfm](#))

[Downloads](#)(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/downloads>)

[A-Z index](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/a-z)(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/a-z>)

ABOUT THE MBS

[News](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/news)(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/news>)

[Fact Sheets](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/factsheet-current)(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/factsheet-current>)

[Subscribe](#)(subscribe.cfm)

[HELP](#)

[FAQ's](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/faqs)(http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/faqs)

MBS interpretation(<http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/Content/MBS-interpretation>)

Follow us

 ([http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/AttachmentsByTitle/rss/\\$FILE/mbsonline.xml](http://www.mbsonline.gov.au/internet/mbsonline/publishing.nsf/AttachmentsByTitle/rss/$FILE/mbsonline.xml))